Pelvic Organ Prolapse (POP)

Prepared by

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Female Genital Prolapse

Introduction

Pelvic organ prolapse is a very common condition, particularly among older women. It's estimated that half of women who have children will experience some form of prolapse in later life, but because many women don't seek help from their doctor the actual number of women affected by prolapse is unknown..

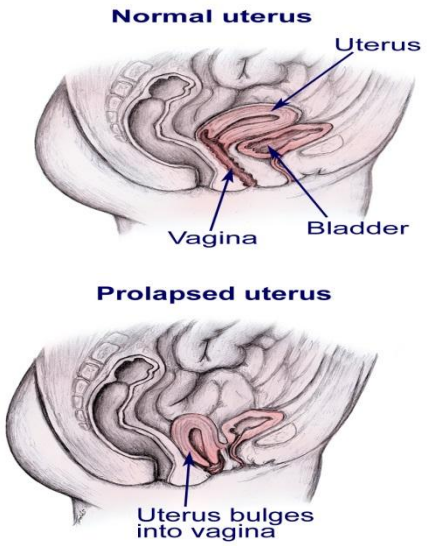
Pelvic floor consists of muscles, fascia and ligaments that form a platform supporting pelvic organs - bladder, uterus and intestines. Pelvic floor prevents these organs from falling down

Normally the uterus lies in ante version & ante flexion. In most women, the long axis of the uterus is bent forward on the long axis of the vagina. This position is referred to as **ante version** of the uterus. Furthermore, the long axis of the body of the uterus is bent forward at the level of the internal os with the long axis of the cervix. This position is termed **anteflexion** of the uterus. Uterus assumes anteverted position in 50% women, retroverted position in 25% women and rest have midposed uterus.

Definition

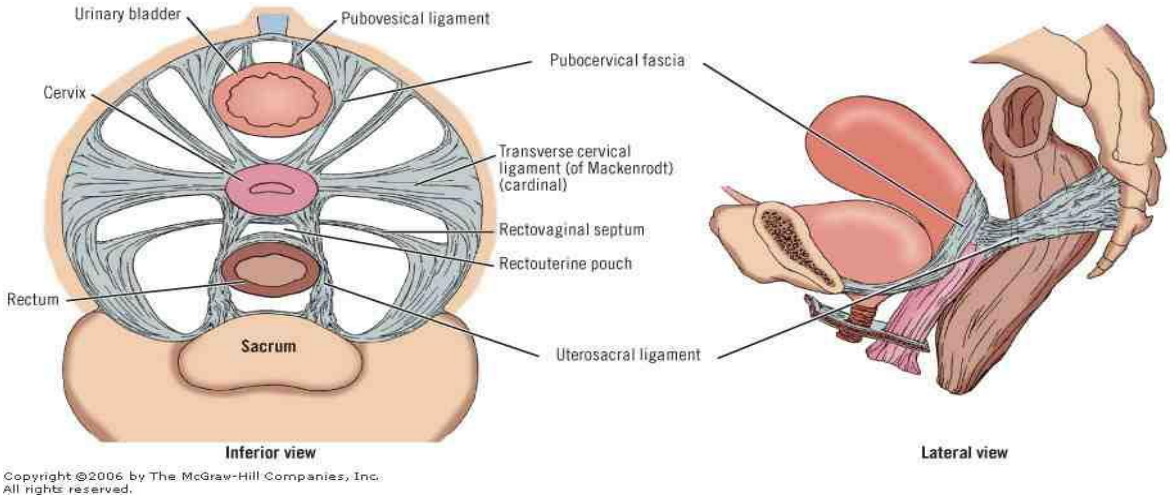
- Genital prolapse is an extrusion or herniation of genital mass through the vagina
- Prolapse means to full out of place.
- Is a condition where organ of the uterus fall down or ship out of place the organs protruding through the vagina or rectum.

• Descent of one or more pelvic structures: the uterine cervix or vaginal apex, anterior vaginal wall (usually with bladder, cystocele), posterior vaginal wall (usually with rectum, rectocele), or peritoneum of the cul-de-sac (usually with small intestine, enterocele).



Major ligaments of the uterus:

It is held in place by several peritoneal ligaments, of which the following are the most important (there are two of each):



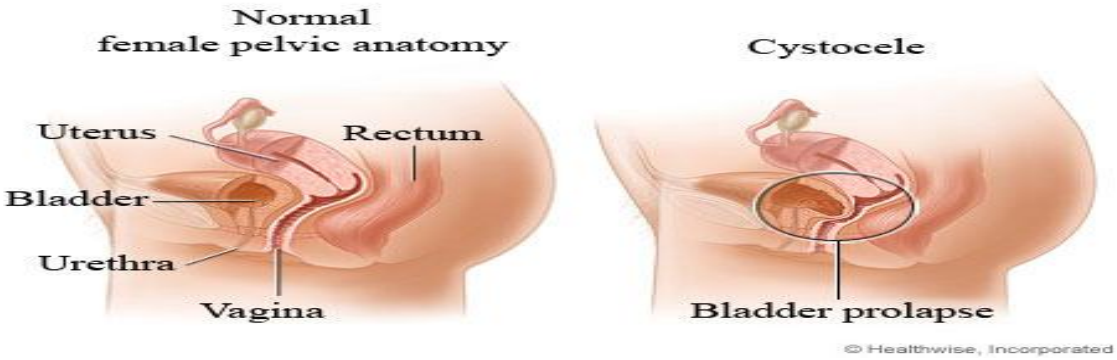
name	*from*	*to*	*function*
Uterosacral ligament	**Posterior cervix**	**Anterior face of sacrum**	**These prevent the uterus from being displaced inferiorly and anteriorly**
Cardinal ligaments	**Side of the cervix**	**Ischial spines**	**These prevent inferior**

			movement of the uterus
Pub cervical ligament	**Side of the cervix**	**Pubic symphysis**	**These restrict posterior movement of the uterus**

Types of pelvic prolapse:

A-prolapse of anterior wall of vagina:

1-***cystocele*** occurs when the posterior bladder wall protrudes downward through the anterior vaginal wall

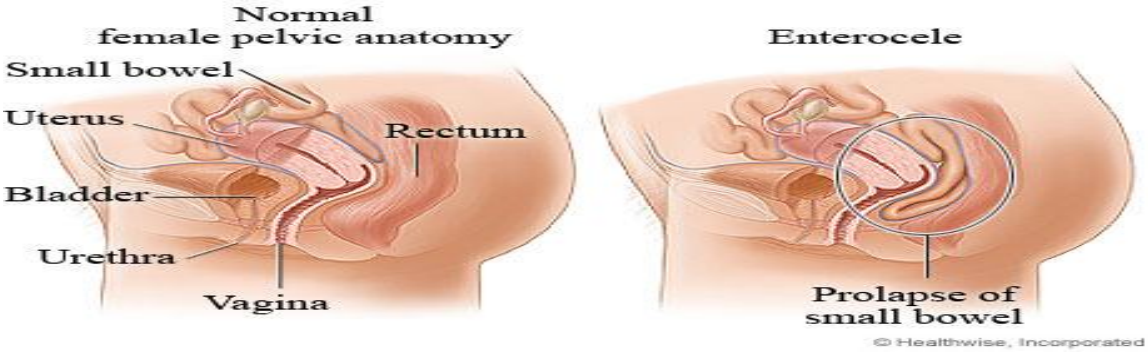


2-***urethrocele*** descent of urethra into vagina

B-Posterior wall vaginal prolapse:

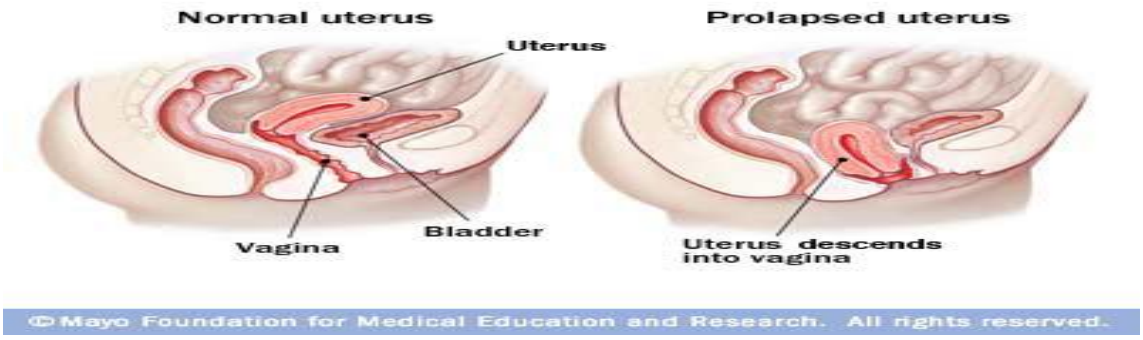
1-***rectocele*** occurs when the rectum sags and pushes against or into the posterior vaginal wall

2-***entrocele*** occurs when the small intestine bulges through the posterior vaginal wall (especially common when straining)

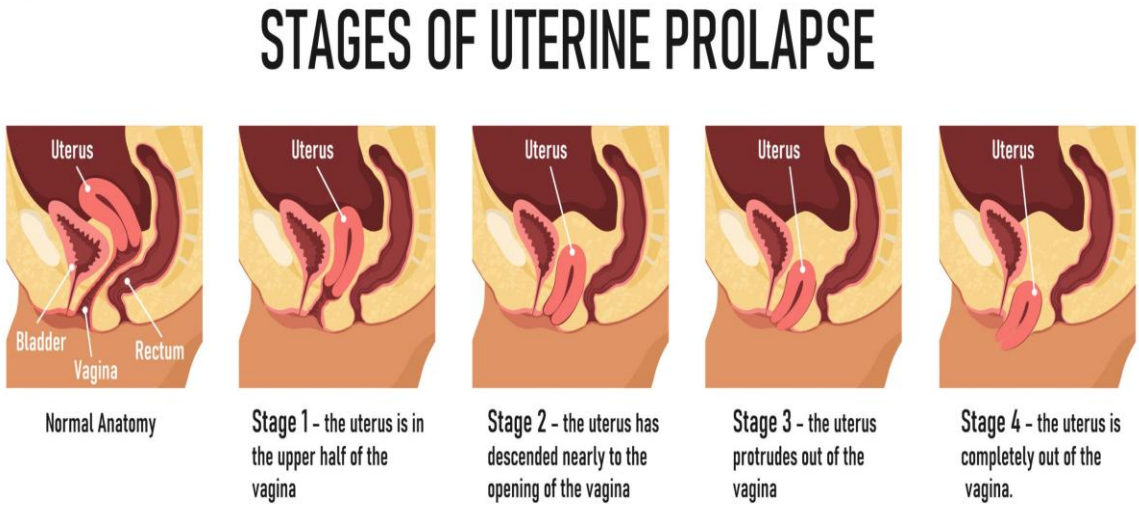


3-***vaginal vault prolapse*** descent of roof of vagina outside the vagina

c-***Uterine prolapse*** occurs when uterus descends through the pelvic floor and into the vaginal canal



Degree of uterine prolapse:



Signs and symptoms:

1-Urinary symptoms:

-stress incontinence
-frequency (diurnal and nocturnal)
-urgency and urge incontinence
-hesitancy
-poor or prolonged stream
-feeling of incomplete emptying

2-bowel symptoms:

-difficulty in defecation
-incontinence of flatus or liquid or solid stool
-urgency of defecation
-feeling of incomplete evacuation
-rectal protrusion or prolapse after defecation

3-sexual symptoms:

-inability to have frequent intercourse
-dyspareunia
-lack of satisfaction or orgasm
-incontinence during sexual activity

4-other local symptoms:

-pressure or heaviness in the vagina
-pain in the vagina
-low back pain after long periods of standing
-prolapse bulge in the vaginal vault
-difficulty walking
-difficulty inserting or keeping a tampon in place

-vaginal cervical mucosa hypertrophy ,excoriation ulceration ,and bleeding
-abdominal pressure or pain

Causes of prolapsed

- Pregnancy and childbirth
- Ageing and the menopause
- Obesity, large fibroids or tumors
- Chronic coughing or strain
- Heavy lifting
- Genetic conditions
- Previous pelvic surgery
- Spinal cord conditions and injury

Treatment:

Prophylactic ttt, Palliative ttt by pessary& (Surgical) treatment

1-Prophylactic treatment:

***Prevention during the ante natal period*:**

(from conception to until birth)

- Education about importance of good ante natal care
- Examination general, local and ultrasound and urine & blood analysis , Good balanced diet - hygiene- rest-sleep-immunization.
- The importance of family planning after birth and exercise.
- Avoid health hazards and treatment chronic anemia.

Prevention during labour

- **First stage :-**Avoid straining. -Bladder & rectum empty.
- **Second stage:** - Avoid voluntary bearing down, Avoid forceps application or breech extraction before full cervical dilation.
- **Third stage**: A void fundal pressure for delivery of the placenta.

***Prevention* during puerperium:**

- Frequent emptying of the bladder.

- Kegel's exercises for the pelvic floor muscle.

- Ring pessary for 2 months of prolapse is diagnosed within 6w.

***Prevention during hysterectomy*:**

By suturing the uterosacral & round ligaments t the vaginal vault.

***Prevention during post natal period*:**

1. Emptying bladder & rectum frequently.
2. Early amputation after labor.
3. Kegel's exercise should be done.
4. Good balanced diet and good hygienic care.
5. Proper family planning.

Prevention during menopasue

1. Oestrogen replacement therapy.
2. Kegel's exercise.
3. Frequency emptying bladder.
4. Good management of infection.
5. Treatment chronic disease which causes ↑ intra abdominal pressure.
6. Good hygienic care & sleep & rest.
7. Avoid sleep in supine position while the bladder is full.

11-palliative treatment :

Physiotherapy

- **Pelvic floor exercises These exercises, also called Kegel exercises, may help:**
- keep the prolapse from getting worse
- Help reduce backache, pelvic pain and incontinence.

- It may take a few months before noticing any improvement.

Hormone Replacement Therapy (HRT)

Hormone replacement therapy may help

- Strengthen the vaginal walls and pelvic floor muscles by increasing the estrogen and collagen levels in body,
- but there is little evidence as to whether it is effective in treating prolapse.

Vaginal Pessaries

- A vaginal pessary is a small device, similar to a diaphragm or cervical cap, which is inserted into the vagina to hold the prolapsed organ(s) in place.
- Pessaries are made of latex or silicone and come in many different shapes and sizes.

- Inserting, removing and cleaning pessary
- Learn to remove and reinsert it properly on your own.
- Remove the pessary and clean it with ordinary soap and water every few weeks (usually 3 weeks). Do not clean the pessary with any other washing liquids.
- On removal, inspect the pessary for cracks or breaks. If there are any it should be replaced.
- Always inspect the vagina for allergic reactions, injuries or infections.
- Do not reinsert the pessary if there is a sore or infection in the vagina.
- Attend regularly scheduled visits to the doctor to have the pessary removed and cleaned and the vagina examined.

Complications of Using a Pessary

- Use of pessaries can cause injuries and ulceration in women with thin vaginal mucosa such as postmenopausal women. Treatment with an estrogen cream before pessary insertion can prevent this.

- Pessaries can cause vaginal discharge and odor. These can be treated by douching with dilute vinegar or using an acidic vaginal gel.
- If a pessary is improperly inserted, it may damage surrounding structures.
- Pessaries also cause irritation and itching due to allergy. This can also be relieved by douching with dilute vinegar or hydrogen peroxide or changing the pessary.
- Sometimes the pessary may become embedded in vaginal mucosa and can be difficult to remove. This is treated by using an estrogen cream. It may require surgical intervention.

Chong, W. and Fantl, J.A. (2020). Pelvic Organ Prolapse. In Obstetrics and Gynecology, R. Sperling (Ed.). https://doi.org/10.1002/9781119450047.ch21

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